Pathcare Labs Pvt. Ltd.
Regional Laboratory - 525A/486/06, Sector-A, 1st Floor
Behind Classic Restaurant, Gole Market, Mahanagar,
Lucknow, Uttar Pradesh-226006
Ph:- 04061216121

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REPORT

Patient Name	:	Mr. ADITYA NARAYAN	Reg. No.	:	02112610270035
Age and Sex	:	53 Yrs / Male	PCC Code	:	PCL-UP-3145
Referring Doctor	:	Self	Sample Drawn Date	:	27-Oct-2026 12:00 AM
Referring Customer	:	N/A	Registration Date	:	27-Oct-2026 02:13 PM
Vial ID	:	W1938617	Report Date	:	27-Oct-2026 06:22 PM
Sample Type	:	*WB-EDTA*	Report Status	:	Final Report
Client Address	:	*Golf City Apt Ansal Lucknow Up*			

HEMATOLOGY

PATHCARE 3.4 PANEL

Test Name	Obtained Value	Units	Bio. Ref. Intervals (Age/Gender specific)	Method
Complete Blood Count (CBC)				
Haemoglobin	15.3	g/dL	13.0-17.0	Colorimetric
RBC Count	4.9	10^12/L	4.5-5.5	Electrical Impedance
Haematocrit (HCT)	45.9	%	40-50	Calculated
MCV	97.9	fl	83-101	RBC Histogram
MCH	31.2	pg	27-32	Calculated
MCHC	32.0	g/dL	31.5-34.5	Calculated
RDW-CV	**17.1**	%	11.6-14.0	RBC Histogram
Platelet Count	**125**	10^9/L	150-410	Electrical Impedance/Microscopy
WBC count, Total	6.3	10^9/L	4.0-10.0	Impedance
Neutrophils	**74.0**	%	40-70	Microscopy
Neutrophil-Absolute Count	4.7	10^9/L	2.0-7.0	Calculated
Lymphocytes	21.0	%	20-40	Microscopy
Lymphocytes-Absolute Count	1.3	10^9/L	1.0-3.0	Calculated
Monocytes	3.0	%	2-10	Microscopy
Monocytes-Absolute Count	0.2	10^9/L	0.2-1.0	Calculated
Eosinophils	2.0	%	1-6	Microscopy
Eosinophils-Absolute Count	0.1	10^9/L	0.02-0.5	Calculated
Basophils	0.0	%	0-2	Microscopy
Basophils-Absolute Count	0.0	10^9/L	0.0-0.3	Calculated
Others	0.0	%	00	Microscopy
Remarks	-		0	

Sample is Processed on Automated CBC Analyzer
Note: *Haematocrit (HCT) is derived from calculated MCV based on RBC Histogram as per Manufacturer's Manual*

Result rechecked and verified for abnormal cases.

***Erythrocyte Sedimentation Rate (ESR)**	**14**	mm in 1hr 12 or less	Westergren method

Comment:
Conditions that may be associated with a highly elevated ESR include the Hypersensitivity Vasculitis, Giant Cell Arteritis, Waldenstrom Macroglobulinemia, Polymyalgia Rheumatic, Metastatic Cancer, Chronic infection, Hyperfibrogenemia etc.

Result rechecked and verified for abnormal cases.

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REPORT

Patient Name	: Mr. ADITYA NARAYAN	Reg. No.	: 02112610270035
Age and Sex	: 53 Yrs / Male	PCC Code	: PCL-UP-3145
Referring Doctor	: Self	Sample Drawn Date	: 27-Oct-2026 12:00 AM
Referring Customer	: N/A	Registration Date	: 27-Oct-2026 02:13 PM
Vial ID	: W1938619, W1938617	Report Date	: 27-Oct-2026 06:27 PM
Sample Type	: *Plasma-Sodium Fluoride-Fasting*	Report Status	: Final Report
Client Address	: *Golf City Apt Ansal Lucknow Up*		

CLINICAL BIOCHEMISTRY

PATHCARE 3.4 PANEL

Test Name	Obtained Value	Units	Bio. Ref. Intervals (Age/Gender specific)	Method
***Glucose-Plasma -Fasting**	**103.8**	mg/dL	Normal < 100 Pre-diabetic 100-125 Diabetic >= 126	Hexokinase

Comments:
-Glucose is the major carbohydrate present in blood. Its oxidation in the cells is the source of energy for the body. Increased levels of Glucose are found in Diabetes Mellitus, Hyperparathyroidism, Pancreatitis and renal failure.
-Decreased levels are found in Insulinoma, Hypothyroidism, Hypopituitarism and extensive Liver disease
Criteria for diagnosis of Diabetes mellitus:
-FPG >/=126 mg/dL, Fasting is defined as no caloric intake for atleast 8 hours.
-Fasting hypoglycemia may be observed in persons taking certain diabetes medications, antibiotics and alcoholic beverages.
-Additionally intestinal disorders, endocrine disorders, insulin surge, nature of diet, reduced food intake than usual and stress are among several other factors. Please correlate clinically.
Reference: American Diabetes association guidelines 2021.

Result rechecked and verified for abnormal cases.

***Glycosylated Hemoglobin(GHb/HbA1c)**	6.10	%	Normal: < 5.7 Pre-Diabetic: 5.7-6.4 Diabetic: >= 6.5	HPLC
***Glycosylated Hemoglobin**	43.17	mmol/mol		Calculated
***Mean Blood Glucose**	**128.37**	mg/dL	90 - 120 : Excellent Control 121 - 150 : Good Control 151 - 180 : Average Control 181 - 210 : Action Suggested >211 : Panic Value	Calculated

Comments:

- HbA1c is an indicator of glycemic control. HbA1c represents average Glycemia over the past six to eight weeks. Glycation of Hemoglobin occurs over the entire 120 day life span of the Red Blood Cell, but within this 120 days. Clinical studies suggest that a patient in stable control will have 50% of their HbA1c formed in the month before sampling, 25% in the month before that, and the remaining 25% in months two to four.
- Mean Plasma Glucose mg/dL = 28.7 x A1C - 46.7. Correlation between HbA1c and Mean Plasma Glucose (MPG) is not "perfect" but rather only this means that to predict or estimate average glucose from HbA1c or vice-versa is not "perfect" but gives a good working ballpark estimate.
- Afternoon and evening results correlate more closely to HbA1c than morning results, perhaps because morning fasting glucose levels vary much more than daytime Glucose levels, which are easier to predict and control. As per IFCC recommendations 2007, HbA1c being reported as above maintaining traceability to both IFCC (mmol/mol) & NGSP (%) units.
- Reference: ADA (American Diabetic Assosciation) Guidelines 2023.

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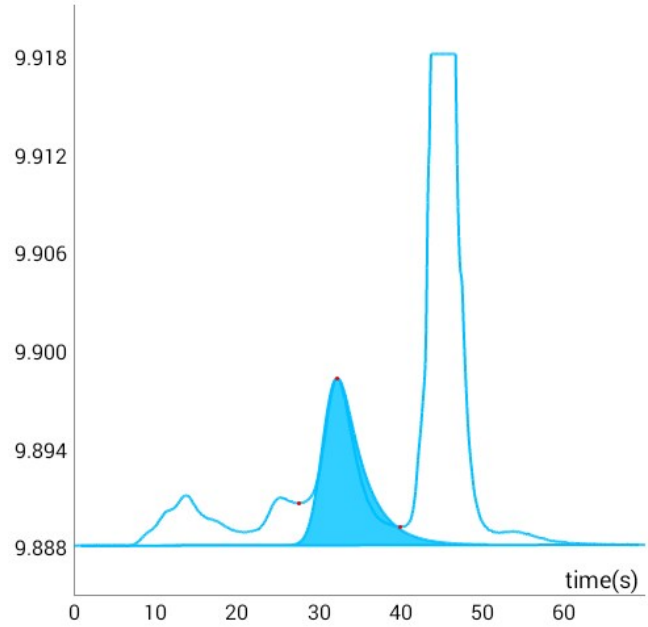
REPORT

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Age and Sex	:	53 Yrs / Male	PCC Code	:	PCL-UP-3145
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CLINICAL BIOCHEMISTRY

PATHCARE 3.4 PANEL

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Referring Customer	:	N/A	Registration Date	:	27-Oct-2026 02:25 PM
Vial ID	:	W1938618	Report Date	:	27-Oct-2026 04:25 PM
Sample Type	:	*Serum*	Report Status	:	Final Report
Client Address	:	*Golf City Apt Ansal Lucknow Up*			

CLINICAL BIOCHEMISTRY

PATHCARE 3.4 PANEL

Test Name	Obtained Value	Units	Bio. Ref. Intervals (Age/Gender specific)	Method
Lipid Profile				
Cholesterol Total	**205.84**	mg/dL	Adult: Desirable<200 mg/dL, Borderline: 200 – 239 mg/dL, High:>240 mg/dL	Enzymatic
Cholesterol HDL	46.58	mg / dL	40 - 60	Direct Homogenous
Cholesterol - LDL	**120.12**	mg/dL	<100 Optimal	Calculated
Cholesterol VLDL	39.14	mg/dL	7-40	Calculated
Non-HDL cholesterol	**159.26**	mg/dL	Optimal < 130	Calculated
Triglycerides	**195.7**	mg/dL	Normal: <150 Borderline High: 150–199 High: 200–499 Very High >500	Glycerol Phosphate Oxidase
Cholesterol Total/Cholesterol HDL Ratio	**4.4**		0 - 4.0	Calculated
Cholesterol LDL/Cholesterol HDL	2.6		0 - 3.5	Calculated

COMMENTS: Therapeutic target levels of lipids as per NCEP – ATP III recommendations:

Total Cholesterol (mg/dL)	<200 - Desirable, 200-239 - Borderline High, >240 - High
HDL Cholesterol (mg/dL)	<40 - Low, >60 - High
LDL Cholesterol (mg/dL)	<100 Optimal, [Primary Target of Therapy], 100-129 - Near Optimal/Above Optimal, 130-159 - Borderline High, 160-189 - High, >190 Very High
Serum Triglycerides (mg/dL)	<150 Normal, 150-199 Borderline High, 200-499 High, >500 Very High

NCEP recommends lowering of LDL Cholesterol as the primary therapeutic target with Lipid lowering agents, however, if Triglycerides remain >200 mg/dL after LDL goal is reached, set secondary goal for non-HDL Cholesterol (total minus HDL) 30 mg/dL higher than LDL goal.

When Triglyceride level is > 400 mg/dL, Friedewald Equation is not applicable for calculation of LDL & VLDL. Hence the calculated values are not provided for such samples.

ATP III Guidelines:

Risk Category	LDL Goal	LDL Level at Which to Initiate Therapeutic Lifestyle Changes (TLC)	LDL Level at Which to Consider Drug Therapy
CHD or CHD RiskEquivalents(10-year risk >20%)	<100 mg/dL	>100 mg/dL	>130 mg/dL (100-129 mg/dL: drug optional)*
2+ Risk Factors (10-year risk <20%)	<130 mg/dL	>130 mg/dL	10-year risk 10-20%: >130 mg/dL 10-year risk <10%:>160mg/dL
0-1 Risk Factor	<160 mg/dL	>160 mg/dL	>190 mg/dL (160-189 mg/dL: LDL-lowering drug optional)

Result rechecked and verified for abnormal cases.

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Vial ID	:	W1938618	Report Date	:	27-Oct-2026 04:25 PM
Sample Type	:	*Serum*	Report Status	:	Final Report
Client Address	:	*Golf City Apt Ansal Lucknow Up*			

CLINICAL BIOCHEMISTRY

PATHCARE 3.4 PANEL

Test Name	Obtained Value	Units	Bio. Ref. Intervals (Age/Gender specific)	Method
Liver Function Test (LFT)				
Bilirubin Total	0.68	mg/dL	0.2-1.2	Diazonium Salt
Bilirubin Direct	0.18	mg/dL	0-0.5	Diazo Reaction
Bilirubin Indirect	0.5	mg/dL	0.2 - 1.0	Calculated
Alkaline Phosphatase (ALP)	80.8	U/L	40-150	Para-Nitrophenyl-phosphate
Aspartate Aminotransferase (SGOT)	31.8	U/L	11-34	NADH w/o P-5'-P
Alanine Transaminase (ALT/SGPT)	25.3	U/L	0-45	NADH w/o P-5'-P
Gamma Glutamyl Transferase (GGT)	33.61	U/L	12-64	L-g-g-3-Carboxy-4-Nitroanilide subs
Protein Total	6.8	g/dL	6.4-8.3	Biuret
Albumin	4.7	g/dL	3.5-5.0	Bromcresol green
Globulin	**2.06**	g/dl	2.5 - 3.8	Calculated
Albumin / Globulin Ratio	**2.3**		1.0 - 2.1	Calculated
SGOT/SGPT Ratio	1	U/U	>2.0 suggests alcoholic liver disease.	Calculated

*Liver function tests are blood tests used to help diagnose and monitor Liver disease or damage. Screen for Liver infections, such as Hepatitis, monitor possible side effects of medications.Measure the severity of a disease, particularly scarring of the Liver (Cirrhosis)
*__Alanine Transaminase (ALT)-__ an enzyme found in the Liver that helps your body metabolize protein. When the Liver is damaged, ALT is released into the bloodstream and levels increase.
*__Aspartate Transaminase (AST)-__ an enzyme that helps metabolize Alanine, an amino acid. Like ALT, AST is normally present in blood at low levels. An increase in AST levels may indicate Liver damage or disease or Muscle damage.
*__Alkaline Phosphatase (ALP)__- an enzyme in the Liver, bile ducts and bone. Higher-than-normal levels of ALP may indicate liver damage or disease, such as a blocked bile duct, or certain bone diseases.
*__Albumin and Total Protein-__ Albumin is one of several proteins made in the Liver. Your body needs these proteins to fight infections and to perform other functions. Lower-than-normal levels of albumin and total protein might indicate Liver damage or disease
*__Bilirubin-__ a substance produced during the normal breakdown of red blood cells. Bilirubin passes through the liver and is excreted in stool. Elevated levels of bilirubin (jaundice) might indicate liver damage or disease or certain types of anemia.
*__Gamma-Glutamyltransferase (GGT)-__ GGT is an enzyme in the blood. Higher-than-normal levels may indicate liver or bile duct damage.

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Sample Type	:	*Serum*	Report Status	:	Final Report
Client Address	:	*Golf City Apt Ansal Lucknow Up*			

CLINICAL BIOCHEMISTRY

PATHCARE 3.4 PANEL

Test Name	Obtained Value	Units	Bio. Ref. Intervals (Age/Gender specific)	Method
Calcium - Serum				
Calcium	9.20	mg/dl	8.4-10.2	Arsenazo III Complex

Interpretation:

Category	Normal Ref. Range
Premature	6.2 mg/dL to 11.0 mg/dL
0 to 10 days	7.6 mg/dL to 10.4 mg/dL
10 days to 24 months	9.0 mg/dL to 11.0 mg/dL
Child 2 to 12 years	8.8 mg/dL to 10.8 mg/dL
Adult	8.4 mg/dL to 10.2 mg/dL
Male > 60 years	8.8 mg/dL to 10.0 mg/dL

Comments:
* Calcium in the body is found mainly in the bones (approximately 99%). In serum, Calcium exists in a free ionised form and in bound form (with Albumin). Hence, a decrease in Albumin causes lower Calcium levels and vice-versa.
* Calcium levels in serum depend on the Parathyroid Hormone.
* Increased Calcium levels are found in Bone tumors, Hyperparathyroidism. decreased levels are found in Hypoparathyroidism, renal failure, Rickets.

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REPORT

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Age and Sex	:	53 Yrs / Male	PCC Code	:	PCL-UP-3145
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Sample Type	:	*Serum*	Report Status	:	Final Report
Client Address	:	*Golf City Apt Ansal Lucknow Up*			

CLINICAL BIOCHEMISTRY

PATHCARE 3.4 PANEL

Test Name	Obtained Value	Units	Bio. Ref. Intervals (Age/Gender specific)	Method
Iron Deficiency Profile-II				
Iron	133	µg/dL	65-175	Ferene
UIBC(Unsaturated Iron-Binding Capacity)	289.60	µg/dL	120–470 µg/dL	Ferrozine
Iron Binding Capacity - Total (TIBC)	422	µg/dL	250-450	Calculated
Transferrin	**295.1**	mg/dL	176 - 280	Calculated
% Iron Saturation	31	%	20 - 50	Calculated

Iron Iron found in the blood is mainly present in the hemoglobin of the RBCs. Its role in the body is mainly in the transportation. Iron is absorbed in the Small Intestine, and bound to a globulin in the plasma called Transferrin and transported to the Bone Marrow for the formation of Hemoglobin. Increased serum levels are found in Hemolytic Anemias, Hepatitis, Lead and Iron poisoning. Decreased serum levels are found in Anemias caused by Iron Deficiency due to insufficient intake or absorption of Iron, chronic blood loss, late pregnancy and Cancer

TIBC The serum TIBC varies in disorders of Iron metabolism. In Iron-deficiency Anemia the TIBC is elevated and the transferrin saturation is lowered to 15% or less. Low serum Iron associated with low TIBC is characteristic of the anemia of chronic disorders, malignant tumors, and infections.

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REPORT

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Vial ID	:	W1938618	Report Date	:	27-Oct-2026 03:42 PM
Sample Type	:	*Serum*	Report Status	:	Final Report
Client Address	:	*Golf City Apt Ansal Lucknow Up*			

CLINICAL BIOCHEMISTRY

PATHCARE 3.4 PANEL

Test Name	Obtained Value	Units	Bio. Ref. Intervals (Age/Gender specific)	Method
Thyroid Profile I				
Tri-Iodothyronine Total (TT3)	97.14	ng/dL	40-181	CMIA
Thyroxine - Total (TT4)	8.46	µg/dL	4.6-10.5	CMIA
Thyroid Stimulating Hormone (TSH)	1.3903	µIU/mL	0.4-4.2(Test performed on 4th Generation TSH kit)	CMIA

Pregnancy			
	TSH(µIU/mL)	**TT3(ng/dL)**	**TT4(µg/dL)**
1 Trimester	0.10-2.50	89.9-196.6	4.4-11.5
2 Trimester	0.2-3.00	86.1-217.4	4.9-12.2
3 Trimester	0.3-3.00	79.9-186	5.1-13.2

Interpretation:

- Assay results should be interpreted in context to the clinical condition and associated results of other investigations.
- Previous treatment with corticosteroid therapy may result in lower TSH levels while Thyroid hormone levels are normal.
- Results are invalidated if the client has undergone a radionuclide scan within 7-14 days before the test.
- Abnormal thyroid test findings often found in critically ill clients should be repeated after the critical nature of the condition is resolved.
- The production, circulation, and disposal of Thyroid hormone are altered throughout the stages of pregnancy.
- **Hyperthyroidism (overactive thyroid):**
  Hyperthyroidism (overactive Thyroid) occurs when your thyroid gland produces too much of the hormone Thyroxine. Hyperthyroidism can accelerate your bodys metabolism, causing unintentional weight loss and a rapid or irregular heartbeat.
- **Hypothyroidism (underactive thyroid):**
  Hypothyroidism (underactive thyroid) is a condition in which your Thyroid gland doesnt produce enough of certain crucial hormones. Hypothyroidism may not cause noticeable symptoms in the early stages. Over time, untreated Hypothyroidism can cause a number of health problems, such as obesity, joint pain, infertility and heart disease.

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CLINICAL BIOCHEMISTRY

PATHCARE 3.4 PANEL

Test Name	Obtained Value	Units	Bio. Ref. Intervals (Age/Gender specific)	Method
***Vitamin - B12**	188.6	pg/mL	187-833	CMIA

Comments:

- Vitamin B12 is essential in DNA synthesis Hematopoiesis, and Central Nervous System integrity.
- Its absorption depends on the presence of intrinsic factor (IF) and may be due to lack of IF secretion by gastric mucosa.
- Vitamin B12 deficiency frequently causes Macrocytic Anemia, Glossitis, Pheripheral Neuropathy, Weakness, Hyperflexia, Ataxia, Loss of Proprioception, poor coordination and effective behavioural changes. A significant increase in RBC MCV may be an important indicator of Vitamin B12 deficiency.

***25 - Hydroxy Vitamin D- Serum**	**26.89**	ng/mL	Deficiency - < 20 Insufficiency - 20 -30 Sufficiency - 30 - 100 Toxicity - >100	CMIA

Comments:

- 25 OH Vitamin D is total of Vitamin D in Bone and mineral metabolism was recognized from its first identification as a factor that could cure rickets. However, Vitamin D is now recognized as a prohormone which has multiple roles in maintaining optimal health.
- Vitamin D toxicity is a recognized problem but a rare occurrence. Instead, a recent growing public health problem is Vitamin D insufficiency.

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CLINICAL BIOCHEMISTRY

PATHCARE 3.4 PANEL

Test Name	Obtained Value	Units	Bio. Ref. Intervals (Age/Gender specific)	Method
Kidney Function Test (KFT) -Extended				
Urea	19.84	mg/dL	10 - 50	Urease
Creatinine	0.81	mg/dL	0.72-1.25	Kinetic Alkaline Picrate
Calcium	9.20	mg/dl	8.4-10.2	Arsenazo III Complex
Uric Acid	5.88	mg/dL	3.5 - 7.2	Uricase
Phosphorus - Serum	3.26	mg/dL	2.3-4.7	Phosphomolybdate
Sodium	138.3	mmol/L	135 - 145	ISE Direct
Potassium	4.56	mmol/L	3.5 - 5.5	ISE Direct
Chloride	102.5	mmol/L	98-107	ISE Direct
Blood Urea Nitrogen (BUN)	9.27	mg/dL	6.0-20.0	Urease
BUN / Creatinine ratio	11.44		0-20	Calculated
Urea / Creatinine Ratio	24.49	mg/mg	Elevated ratio: >100:1 Reduced ratio: <40:1	Calculated
GFR by MDRD Formula	99.68	mL/min/1.73m2	74 - 129	

Kidney blood tests, or kidney function tests are used to detect and diagnose disease of the kidney

The higher the blood leavels of urea and creatine ,the less well the kidneys are working
the level of creatine is usually used as a marker as to the severity failure(Creatine in itself is not harmful but a high level indicates that the kidneys are not working properly,somay other waste products will not be cleared out of the bloodstream) you normally need treatment with dialysis if the leavel of creatine goes higher then a certain valu.
Dehydration can also be a come for increases in urea level.
Before and after starting trearment with certain medicines .some medicines occasionally cause kidney damge(Nephrotoxic Drug)as a side-effect.Therefore Kidney functionos often checked before and after starting treatment with certain medicines

Dr.MOHIT
MBBS,MD Pathology
UPMCI Reg.No.47041

Note: If the test results are alarming or unexpected,Client is advised to contact the laboratory immediately for possible remedial action.
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Pathcare Labs Pvt. Ltd.
Regional Laboratory - 525A/486/06, Sector-A, 1st Floor
Behind Classic Restaurant, Gole Market, Mahanagar,
Lucknow, Uttar Pradesh-226006
Ph:- 04061216121

BARCODED | NETWORKED | ACCREDITED

REPORT

Patient Name	:	Mr. ADITYA NARAYAN	Reg. No.	:	02112610270035
Age and Sex	:	53 Yrs / Male	PCC Code	:	PCL-UP-3145
Referring Doctor	:	Self	Sample Drawn Date	:	27-Oct-2026 12:00 AM
Referring Customer	:	N/A	Registration Date	:	27-Oct-2026 03:06 PM
Vial ID	:	W1938620	Report Date	:	27-Oct-2026 03:18 PM
Sample Type	:	*Urine*	Report Status	:	Final Report
Client Address	:	*Golf City Apt Ansal Lucknow Up*			

CLINICAL PATHOLOGY

PATHCARE 3.4 PANEL

Test Name	Obtained Value	Units	Bio. Ref. Intervals (Age/Gender specific)	Method
Complete Urine Analysis (CUE)				
PHYSICAL EXAMINATION				
Colour	Pale Yellow	-	Straw to light amber	Visual Examination
Appearance	Clear	-	Clear	Visual Examination
CHEMICAL EXAMINATION				
Glucose	Negative	-	Negative	Glucose Oxidase
Protein	(+)	-	Negative	Protein-error-of-indicator
Bilirubin (Bile)	Negative	-	Negative	Diazonium azo
Ketone Bodies	Negative		Negative	Nitroprusside
Specific gravity	1.020		1.001 - 1.035	Hydrogenous lonogen
Blood	Negative		Negative	Peroxidase
Reaction (pH)	6.5		4.6 - 8.0	pH Indicator
Nitrites	Negative		Negative	Diazonium
Leukocytes	Negative		Negative	Pyrrole
MICROSCOPIC EXAMINATION				
PUS(WBC) Cells	**8-10**	/hpf	00-05	Microscopy
Red Blood Cells	Absent	/hpf	Absent	Microscopy
U.Epithelial Cells	2-4	/hpf	00-05	Microscopy
Casts	Absent	/hpf	Occasional Hyaline cast	Microscopy
Crystals	Absent	/hpf	Absent	Microscopy
Others	Absent	/hpf	Absent	Microscopy

Result rechecked and verified for abnormal cases.

Correlate Clinically.

*** End Of Report ***

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